Allocation Details	
Allocation Title	**Multi-sectoral humanitarian assistance to newly placed IDPs, people living within frontline areas and areas affected by ongoing shelling**
Allocation Type and Round	2025 First Standard Allocation
Allocation Amount	**$70,000,000**
Emergency Type	Conflict-related Crisis
Emergency Sub-types	Violence /Clashes, Displacement
Allocation Launch Date	23 December 2024 **– 18:00 Kyiv Time**
Project Proposal Submission Opens in OneGMS	17 January 2025 **– 10:00 Kyiv Time**
Proposal Submission Deadline	07 February 2025 **– 18:00 Kyiv Time**

Section 1: Strategic Statement

Through this allocation, the UHF aims to support comprehensive, vulnerability-focused, and needs-driven multi-sectoral assistance to newly placed IDPs, returnees and people living within frontline areas and areas affected by ongoing shelling, aligning with prioritized activities under the 2025 Humanitarian Needs and Response Plan (HNRP). Priority must be given to extremely vulnerable individuals and locations of highest severity – please see below.

In particular, the allocation targets support and essential services for 1) vulnerable newly displaced individuals from frontline areas, 2) people living in frontline areas and areas affected by ongoing shelling, and 3) strengthening the capacity of smaller Women's Rights Organizations (WROs), organizations of people with disabilities (OPDs), and organizations representing Roma and LGBTIQ+ communities.

The allocation will reinforce the UHF's continued commitment to localization by fostering the participation and capacity strengthening of Ukrainian organizations, including national and local NGOs, Civil Society Organizations (CSOs), and volunteer groups capable of delivering principled, context-specific last-mile assistance, particularly in hard-to-reach areas. Specifically, the allocation will support local women's rights organizations and organizations supporting marginalized groups under strategic objective 3.

Section 2: Humanitarian Context

Since the escalation of the war in February 2022, an estimated 3.7 million people remain internally displaced, with over 78,000 of the most vulnerable housed in nearly 1,800 collective sites across 24 oblasts, constituting around 2 per cent of the displaced population.

In 2024, hostilities intensified significantly, with the length of the front line in Ukraine expanding by 15 per cent, notably in northern Kharkivska, while the intensity of violence doubled in Sumska and Chernihivska oblasts. Oblasts like Donetska, Kharkivska and Zaporizka have been severely impacted, particularly in front line raions such as Bakhmutskyi, Pokrovskyi, Kupianskyi and Polohivskyi, which experienced widespread airstrikes and artillery attacks. Between May and November, the most significant incidents impacting civilians and leading to mass evacuations occurred in Kharkivska, Donetska, and Sumska oblasts. In Donetska oblast, authorities evacuated families with children from over 40 towns and villages between August and September, significantly reducing front-line town Pokrovsk's population from 50,000 to 10,000.

With expanded Government-led evacuation mandates, more people continue to be evacuated and displaced from and within the east and north. However, some of the most vulnerable people, including children, single female care providers, older people and those with disabilities, have remained in areas with active hostilities. Meanwhile, an

estimated 12.5 million people who were not displaced from their homes, but are directly affected by the war, are among the most vulnerable with heightened needs due to limited access to services. This includes those who return to their places of origin. Although these people are spread throughout Ukraine, the highest concentration is in front-line regions and areas along the border with Russia.

Humanitarian needs in Ukraine remain critical, affecting civilians in every region. Compared to 2024, extreme and catastrophic levels of need persist in front-line areas, while extreme needs have expanded within northern areas due to the intense impact of the intensified hostilities. Intersectoral catastrophic conditions are occurring in Donetska, Dnipropetrovska, Kharkivska, Khersonska, Luhanska, Zaporizka, with sector specific catastrophic conditions also occurring in Cherkaska, Mykolaivska, and Sumska. Overall, there has been an average increase in the severity of humanitarian conditions across all regions, except for Kyiv City and Kyivska oblast, where there has been a small reduction in the overall severity of needs. It should be recognized that vulnerable groups are more likely to face higher severity levels, often with multiple overlapping needs.

The number of people in need (PiN) of humanitarian assistance is estimated to be 12.7 million, including 2 million children, with 2.8 million internally displaced people in need of assistance and 9.9 million people who are non-displaced but have been more acutely affected by the war, including those who have been previously displaced and returned to their homes. The concentration of PiN remains high in the oblasts in the east, north, and south regions.

In anticipation of continued hostilities in 2025, the impact of the war is expected to continue and further exacerbate the humanitarian situation and correlated needs, with subsequent displacement, damage to critical civilian infrastructure, and disruption of essential services. The battle for territorial control is characterized by escalations and intensified hostilities, which creates a fluid situation. This will likely lead to new waves of displacement and mandatory evacuations, particularly in the front-line oblasts, such as Donetska, Kharkivska, and Zaporizka.

To respond to the identified needs, humanitarian organizations in Ukraine under the 2025 HNRP will aim to support 6 million internally displaced people and non-displaced war-affected people, including returnees – with two Strategic Objectives (SOs):

- **SO1: Life-saving Emergency Assistance.**
- **SO2: Access to Prioritized Essential Services.**

Section 3: Allocation Objective and Thematic Priorities

This allocation will support the Ukraine 2025 HNRP through response prioritization guided by rigorous inter-sectoral severity analysis, supported by the 2024 MSNA data, other cluster-specific and thematic assessments, as well as the 2024 HNRP mid-year response and gap analysis, and complement the Government's response. Priorities have been formulated in consultation with partners and the ICCG.

The overall objective of the allocation is to provide comprehensive vulnerability-focused and needs-driven multi-sectoral humanitarian assistance to vulnerable IDPs, returnees, and other conflict-affected people in Ukraine.

The allocation will focus on three thematic priorities to address the most pressing humanitarian needs of vulnerable people in areas where the severity levels are the highest:

1. **Support to vulnerable newly displaced people from frontline areas.**

   Under this priority, the UHF will fund a multi-sectoral "Response Package" for newly displaced people from frontline areas. UHF partners will provide evacuation support and limited time-bound assistance in collective sites in receiving areas as designated by the government of Ukraine or receiving high numbers of self-evacuees. Response in collective sites in oblasts most impacted by new evacuee arrivals in frontline and border areas, will be complemented by a critical package of support services for evacuees in collective sites in West and Central Ukraine, in line with the agreed ICCG strategy.

2. **Provide prioritized essential services for people living within frontline areas and areas affected by ongoing shelling.**

   Under this priority, partners should provide multi-sector humanitarian assistance to address needs as a result of the direct impact of the ongoing war and allow for more flexible programming to respond to new emergencies. Activities should enable access to services through existing national mechanisms to the extent possible, while complementing Ukraine's shock-responsive social protection system, adhering to the "Reinforce, Do Not Replace" principle outlined in the Agenda for Humanity.

3. **Strengthen the capacity of smaller Women's Rights Organizations (WROs), organizations of People with Disabilities (OPDs), and organizations representing Roma and LGBTIQ+ communities.**

   Under this priority, partners should seek to strengthen the capacity of smaller local Women's Rights Organizations (WROs), organizations of people with disabilities, and organizations representing Roma and LGBTQI+ populations, in order to expand the pool of UHF-eligible Ukrainian organizations with expertise in providing gender-responsive and inclusive humanitarian assistance.

Section 3.1: Strategic Focus

This allocation aims to support the following strategic focus in partner programming to achieve the above thematic priorities:

- **Reaching the most vulnerable people**, including IDPs, returnees, and other conflict-affected people with severe, compound humanitarian needs, in particular persons with disabilities, older people, vulnerable women and children, marginalized groups, including Roma and LGBTIQ+ communities, and people affected by chronic conditions.
- **Localization:** Direct submissions from eligible national NGOs, including women-led and women rights organizations, and submissions from eligible international NGOs with sub-implementing partnerships with national and local partners, including CSOs and volunteer groups providing last-mile humanitarian assistance in targeted communities, will be prioritized. For the latter, the main partners leading the submission should include appropriate capacity support for the sub-implementing partners as well as monitoring activities for quality control and risk assurance in the project log frame and budget these activities accordingly.

  The UHF promotes the following good practices in local partnerships:

  - Tailored and well-timed capacity-building support to local partners;
  - Mandatory provision of duty of care package to front line responders, including first aid and security training, mental and psychosocial support (MHPSS), personal protective equipment for operations in high-risk areas, and life and accident insurance;
  - Mandatory fair sharing of PSC with local partners based on their operational coverage;
  - Sufficient sub-grant duration to allow local partners' organizational continuity and development (six months or longer is recommended);
  - Innovations promoting strategic and effective localization; and
  - Making funding to local partners as direct as possible (no sub-sub-granting).

  In the event that the main partner sub-contracts national or international NGOs, CSOs, CBOs or volunteer groups (Vs), the partnership arrangements and sub-grants shall be clearly spelled out in the project proposal. The main partner shall be fully responsible for all work performed by such sub-contractors and for all acts and omissions committed by them, their employees, or other personnel engaged by them.

- **Disability inclusion:** Projects demonstrating how the project aims to reduce discrimination and barriers for persons with disabilities to fully engage with and benefit from the response (including needs assessments, log

frame, monitoring, and accountability mechanisms with appropriate resources budgeted) will be prioritized. The inclusion of one or more disability standard indicators in the log frame is strongly encouraged.

- **Protection Mainstreaming:** All projects should incorporate protection principles and promote meaningful access, safety, and dignity in humanitarian aid, including the prevention, mitigation and response to gender-based violence. Partners are required to specifically mention how protection mainstreaming will be integrated into their proposed response. Partners are encouraged to use Protection Cluster and AoRs tools to facilitate referral of people in need of protection assistance using Minimum Standards for Individual Protection Referrals and Service Advisor mapping platform to identify relevant service providers. Additionally, the use of Child Protection AoR referral pathways and CPIMS+ for child protection case management is strongly encouraged.
- **Gender and age considerations**: Specific needs of different gender and age groups and response to address them should be articulated in the project design and implementation plan (including needs assessments, log frame, monitoring, and accountability mechanisms with appropriate resources budgeted). Partners are encouraged to include the needs of children, according to the Minimum Standards for Child Protection in Humanitarian Actions (2019 edition) in their proposals. All projects must include the Gender and Age Marker as per IASC guidelines annexed.
- **Accountability to affected populations (AAP):** Projects demonstrating the involvement of affected people in all phases of the project cycle and accessibility of collective feedback and complaints mechanisms for affected people across gender, age, disability, and other diversity factors, in line with the 2022 Ukraine AAP Action Plan, will be prioritized. The project log frame should include at least one AAP standard indicator.
- **Prevention of sexual exploitation and abuse (PSEA):** Projects demonstrating that the partner (and any sub-implementing partners) has mechanisms in place to prevent, detect, report, and manage incidents of SEA, including assistance to survivors, in line with the 2024 Ukraine PSEA Action Plan, will be prioritized. The project log frame should include at least one PSEA standard indicator.

Section 3.2: Multi-Sector Priorities and Parameters

The allocation will promote the following thematic priorities, aligning with prioritized activities under the forthcoming 2025 HNRP. **Partners are strongly encouraged to carefully review the below and consult relevant clusters when designing project proposals.**

Allocation Priority One - Support to vulnerable newly displaced people from frontline areas

The ongoing hostilities in frontline areas have left the elderly, children and families in highly vulnerable positions and are causing widespread displacement. The most acute challenges persist for the evacuation of people with disabilities and low mobility, older people, and chronically ill people who rely almost exclusively on humanitarian actors' support for evacuations and access to specialized services upon arrival to reception areas.

Most partners have limited resources to provide the required, specialized support en route, and are thus unable to prioritize the most vulnerable groups for evacuation. Very limited specialized accommodation basic service options to host these highly vulnerable individuals and groups in the receiving regions is highlighted in the Ministry of Social Policy's Strategy on evacuating older people and people with disabilities from institutions in the frontline areas.

The UHF, under priority one, will fund a multi-sectoral "Response Package" for newly displaced people from frontline areas. UHF partners will provide evacuation support and limited time-bound assistance in collective sites in receiving areas as designated by the government of Ukraine or receiving high numbers of self-evacuees. Recognizing the high vulnerability of evacuees relocated from frontline and border areas, response in collective sites in oblasts most impacted by new evacuee arrivals in frontline and border areas will be complemented by a critical package of support services in collective sites receiving highly vulnerable evacuees in West and Central Ukraine, in line with the ICCG strategy.

Where possible and appropriate, partners should consider the provision of multi-purpose cash (MPCA) to meet the needs of targeted vulnerable groups. Where the use of MPCA is not feasible, the ICCG has agreed an emergency package of support to meet these needs that includes the most prioritized activities from the 2025 HNRP.

Allocation Priority Two – Provide prioritized essential services for people living within frontline areas and areas affected by ongoing shelling.

In 2024, hostilities intensified significantly, with the length of front line in Ukraine expanding by 15 per cent, notably in northern Kharkivska and the intensity of violence doubling in Sumska and Chernihivska oblasts. Oblasts like Donetska, Kharkivska and Zaporizka have been severely impacted, particularly in front line raions such as Bakhmutskyi, Pokrovskyi, Kupianskyi and Polohivskyi, which experienced widespread airstrikes and artillery attacks.
Critical infrastructure across the front-line areas in Ukraine has been massively damaged or destroyed, with 27 per cent of electricity supply stations, 25 per cent of water facilities, 42 per cent of health-care facilities and 58 per cent of educational facilities impacted. Additionally, systematic attacks impacting transport and water infrastructure have disrupted supply chains, reducing access to essential goods and services in many areas. Health and education services are also severely impacted. By mid-2024, 827 healthcare facilities were fully or partially damaged and about 13 per cent of health facilities had faced significant disruptions in power, water or heating.

Under this priority partners should provide multi-sector humanitarian assistance to address needs as result of direct impact of the ongoing war and allow for more flexible programming to respond to new emergencies. Activities should enable access to services through existing national mechanisms to the extent possible, while complementing Ukraine's shock-responsive social protection system, adhering to the "Reinforce, Do Not Replace" principle outlined in the Agenda for Humanity, mainstream protection and apply risk mitigation measures to promote Do-No-Harm approach.

As under the above priority one, where possible and appropriate partners should consider the provision of multi-purpose cash (MPCA) to meet the basic needs of targeted vulnerable groups. Where the use of MPCA is not feasible, partners shall consider other activities part of the ICCG-agreed emergency package of support to meet these needs that includes the most prioritized activities from the 2025 HNRP. This may include complementarity cash assistance to meet needs outside basic needs (ex: CVA for health, protection, GBV survivors and those at risk, rental support).

Allocation Priority 3

Capacity development is a key component of the UHF's Localization approach and a critical element of its support to local actors. In the context of Ukraine where smaller CSOs are leading the response along the front lines UHF has been increasing support to smaller CSOs and volunteer organizations. Building on lessons learned and best practices developed under previous allocations that supported local CSOs along the front lines, the UHF under this allocation will prioritize support to smaller rights-based organizations.

UHF partners under this priority should aim to strengthen the capacity of smaller local Women's Rights Organizations (WRO's), organizations of people with disabilities, and organizations representing Roma and LGBTIQ+ populations, in order to expand the pool of UHF-eligible Ukrainian organizations with expertise in providing gender-responsive and inclusive humanitarian assistance.

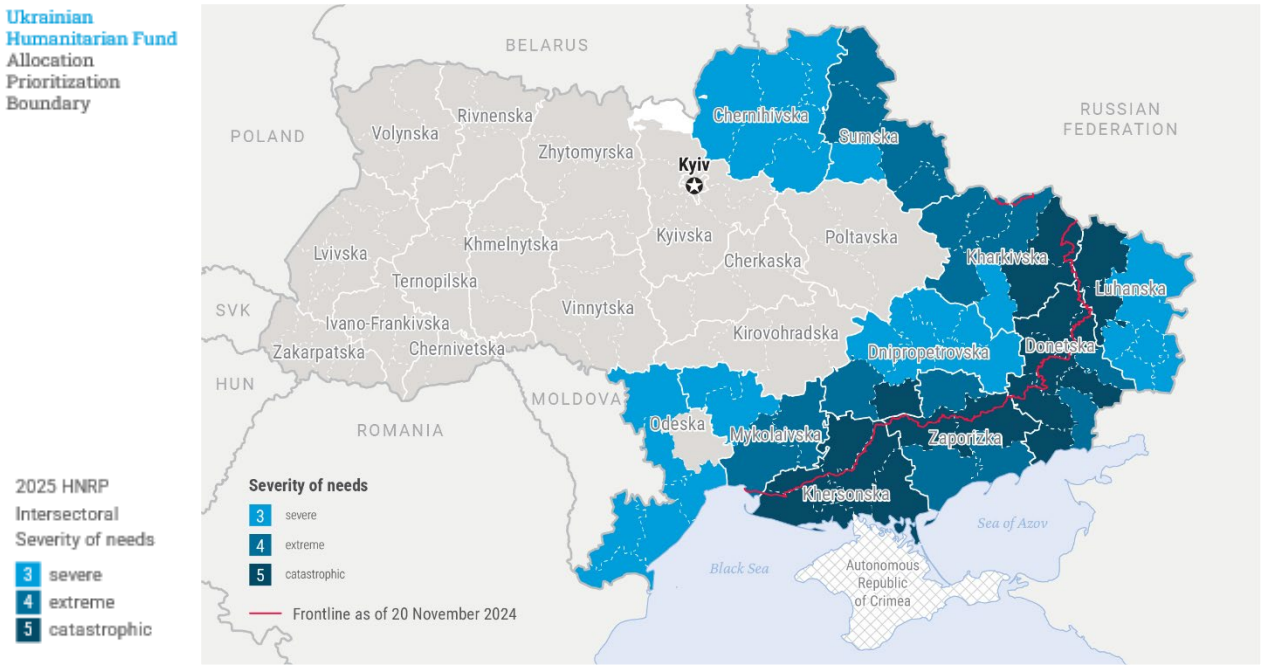
Section 3.3: Allocation Themes and Priority Activities

PRIORITY AND ACTIVITY NAMES	ALLOCATION PRIORITY
Safe and dignified shelter and accommodation	
• Support safe and participatory site management and governance structures	1 & 2
• Site-level monitoring and referrals	1 & 2
• Essential items and equipment for communal and individual use	1 & 2
• Refurbishment of collective sites	1 & 2
• Emergency Shelter Support	1
• Humanitarian Repair	2
• Rental support	1 & 2
• Community led activities	2
• Water & waste-water systems Operation & Maintenance (O&M) and rehabilitation	2
Food and livelihood needs	
• Distribution of in-kind food assistance – hot meal upon arrival at receiving centers	1
• Provision of agricultural inputs (such as seed, fertilizers, tools)	2
• Provision of productive animal survival (such as feed, barns repair and veterinary support)	2
• Protection of emergency off-farm livelihoods in urban areas	2
Health and Hygiene needs of the most vulnerable	
• Improve access to comprehensive, quality and integrated health care including Emergency Medical Services (EMS)	1 & 2
• CVA for health to reduce access barriers to health care	1 & 2
• Support accessible RCCE, and health promotion IEC to improve health and nutrition	1 & 2
• Mental health care for acute stress, anxiety, depression and severe psychological disorders such as PTSD due to recent exposure to violence in health facilities and in community settings	1 & 2
• Procure, pre-position, and distribute essential medicines, equipment and supplies	1 & 2
• Engage in capacity building for health care providers, first responders, and other community members	1 & 2
• WASH NFI distribution - hygiene kits in kind	1
• District Heating systems repairs and rehabilitation	2
• WASH in Institution (rehabilitation to institutions like schools, hospitals, and collective centers)	2
• Emergency water supply	1
• Winter and disease outbreak response, to enable access to essential, lifesaving health services during emergencies and shocks	2
Protection needs of the most vulnerable including the elderly, women and children	
• Transportation and social accompaniment	1 & 2
• Psychosocial support	1 &2
• Case management and home-based care for older people and people with disabilities	1 & 2
• Individual Protection Assistance (cash)	1 & 2
• Legal assistance and HLP legal assistance	1 & 2
• Institutional support (focused only on creation of reception capacity for evacuated people with disabilities in need of facility-based care)	1
• Community-based protection activities	1 & 2
• Provision of Mental Health and Psychosocial Support (MHPSS) Services to Girls and Boys plus parents/caregivers (Individual and Group)	1 & 2
• Child protection awareness raising, risk mitigation, prevention of family separation and dissemination of life-saving information on child protection services and referrals	1 & 2
• Child Protection Case Management	1 & 2

• Emergency case management fund	1 & 2
• GBV Case Management	1 & 2
• Service for children affected by, or at-risk victims of, explosive ordnance (EO)/victim assistance	1 & 2
• Child Protection/Child Friendly Legal Assistance	1 & 2
• Referral to Specialized Services to address the critical needs of children and caregivers	1 & 2
• Family Tracing and Reunification	1 & 2
• Supporting unaccompanied children or children at risk in alternative family-based care.	1 & 2
• Provision of dignity kits to GBV survivors and those at-risk	1
• Cash and voucher assistance through specialized care for GBV survivors and those at-risk	1 & 2
• Psychosocial support services for GBV survivors and those at-risk	1 & 2
• Supporting Safe spaces for GBV service Provision (crisis rooms, day care services, Women and Girls Safe Spaces (WGSS)	1 & 2
• GBV awareness raising, risk mitigation and dissemination of life-saving information on GBV services and referrals	1 & 2
• Explosive Ordnance Risk Education (EORE)	2
Educational needs of children	
• Provision of MHPSS, Social Emotional Learning (SEL), Psychological First Aid (PFA) for children in education spaces	1 & 2
• Provision of learning kits to children (individual, including home-based learning)	1 & 2
• Equipping teachers, as first responders, on education in emergencies (wellbeing, PFA, SEL, with referral to CP and GBV)	2
• Provision of educational kits (ECCD, recreational, shelter kits, learning materials) and essential equipment to shelters in schools and kindergartens (institution)	2
• Temporary Learning Spaces (TLSs) and Establishing Digital Learning Centers (DLCs) to support access to safe learning	2
• Provision of catch-up learning for war-affected children	2

Section 3.4: Allocation Locations

Prioritizing areas of high severity



Partner projects should include raions with severity of high 3 plus 4 and 5 from Oblasts along the frontline – please see map below. For priority reception centers and receiving sites, partners should contact the CCCM Cluster who will

identify on a rolling basis, in response to need and evacuations in real-time (response within 24-48 hours of new evacuee arrival and confirmation of needs).

Section 4: Complementarity

This Standard Allocation will complement ongoing UHF projects where relevant and ensure operational coordination and programmatic synergies among the partners funded. Further, this allocation will complement other donor initiatives supporting the HNRP and complementary programming for transition to recovery to avoid overlaps in assistance and maximize their collective impact.

Section 5: Project Selection Criteria

In addition to supporting the above-mentioned strategic and operational priorities, all projects to be funded through this allocation should pursue the following cross-cutting principles and key programmatic considerations.

Access and operational capacity: Only submissions by partners with demonstrated operational presence/access in identified areas of high severity and capacity to absorb the allocated funds and implement prioritized activities in line with the sectoral technical requirements will be recommended for the Strategic Review. For this, partners should have consulted and reported their activities to relevant clusters before submitting a proposal under this allocation.

Evidence-based approach: Needs are clearly identified and articulated and supported by beneficiary data disaggregated by gender, age group, type, disability, and other factors contributing to the need for specialized assistance. Partners aiming to cover hard-to-reach areas with limited needs data should include in the proposed response planned assessments and relevant justification, in consultation with Clusters.

Cost effectiveness: The budget is fair, proportionate in relation to the context. Proposals demonstrating better cost effectiveness will be prioritized, where: a) for comparable activities and outputs, the total cost is less; b) cost per assisted person is competitive and within an acceptable range for the cluster (based on activity-based costing and cluster-specific standard costs); c) the level of support and personnel costs is reasonable and in line with a given type of implementation modality; and d) the proposed period of implementation is adequate and represents best use of resources at that time.

Monitoring: A realistic monitoring and reporting plan is incorporated in the project to measure the achievements against planned outputs and timeline and the quality of assistance provided, including through sub-implementing partners.

Risk management: Assumptions and risks are comprehensively and clearly spelled out, along with risk mitigation and management strategies.

Coordination and complementarity: Demonstrating the partner's coordination with relevant clusters and key stakeholders (including local authorities and other humanitarian partners in the project location), as well as complementarity between the proposed project and activities implemented with other funding sources.

Section 6. Eligibility criteria for this Allocation

Partner Eligibility

- Partners planning to submit proposals under this allocation must notify and seek guidance from Clusters/Working Group Coordinators of the sectoral components of their planned projects. Projects from partners who have not notified the relevant clusters will not advance to the pre-strategic review stage. The current list of the UHF-eligible partners is located here.
- Only current UHF-eligible partners can apply for direct funding, while non-assessed partners may be sub-implementing partners in submissions led by an eligible partner.
- To align with localization efforts and the Humanitarian Coordinator's endorsement, the UHF funding is to be prioritized for national partners, **excluding direct allocations to UN agencies.**

- Partners with their Due Diligence (DD) page unapproved will not be able to see the open allocation in OneGMS. Eligible partners planning to submit a proposal under this allocation are requested to submit their DD swiftly for HFU review and approval.
- Partners with outstanding compliance issues are not eligible to apply for this allocation. Proposals from these partners will not make it to the Strategic Review (SR) stage and will be rejected.
- **Partners must perform due diligence on suppliers who participate in their procurement processes to ensure that they are not subject to a successful criminal prosecution in the past 5 years.** Partners who have identified suppliers involved in confirmed incidents of corruption or fraud are kindly requested to share this information with UHF by emailing ocha-uhf@un.org and keeping nduta@un.org and roman.oleksiienko@un.org in copy. This information will contribute to forming a public database of suppliers which should not be used in projects implemented through UHF funding. The UHF Excluded Suppliers Database (available under the link) is updated and shared on a regular basis.

Project Duration, Modality and Budget

- Under this allocation, each partner may submit only one multi-sectoral project proposal, including one or more thematic priorities.
- Under this allocation, all projects should complete implementation within 12 months.
- Partners onboarded through a pilot Contextualized Capacity Assessment may apply for funding up to US$250,000 (those with existing grants with final reports submitted or not received grants yet).
- For partners onboarded through full Capacity Assessment, project budget ceiling, disbursement arrangements, and monitoring and reporting requirements for the projects to be funded under this allocation will follow the UHF Operational Modalities below:

Risk Level	Project duration *(months)**	Project value *(thousand USD)*	Max. amount per project *(thousand USD)*	Disbursements *(in % of total)*	Financial reporting			Narrative reporting		Monitoring		Audit
					For disbursements	31-Jan (Calendar year reporting)	Final	Progress	Final	Project monitoring	Financial spot-check	
NGOs and RC/RCs												
High	Less than 6 months	≤ 250	-	60-40	Yes	Yes	Yes	1 mid	Yes	1	1	As per plan
		> 250	600	50-50	Yes	Yes	Yes	1 mid	Yes	1	1	
	6 to 12 months	≤ 400	-	50-50	Yes	Yes	Yes	1 mid	Yes	1	1	
		> 400	1,000	40-40-20	Yes	Yes	Yes	2	Yes	1**	1**	
Medium	Less than 6 months	≤ 500	-	100	-	Yes	Yes		Yes	-	-	
		> 500	1,500	80-20	Yes	Yes	Yes	1 mid	Yes	1	-	
	6 to 12 months	≤ 1,000	-	100	-	Yes	Yes	1 mid	Yes	1	-	
		> 1,000	2,500	80-20	Yes	Yes	Yes	1 mid	Yes	1	1	
Low	Up to 12 months	≤ 2,000	-	100	-	Yes	Yes	1 mid (if $1M or above)	Yes	-	-	
		> 2,000	5,000	80-20	Yes	Yes	Yes	1 mid	Yes	-	-	

The UHF reserves the right to conduct additional ad-hoc monitoring or FSC of any partner projects.

* Maximum project duration is 12 months. Cost and No-Cost Extensions may not extend the duration of a project beyond 24 months. If following a Cost or No-Cost Extension additional monitoring and reporting is required in line with the Operational Modalities, the corresponding changes will be made in the OneGMS to ensure oversight activities are conducted.

** An additional monitoring/FSC may be added in case the project is extended beyond 15 months.

Section 7: Process and Timeline

7.1 Allocation Strategy Development Process

The allocation round uses the standard allocation modality, with the strategic scope and prioritization criteria developed based on the 2025 HNRP. Relevant cluster inputs and consultations within OCHA informed the proposed demographic and geographic targeting, as well as operational and cross-cutting priorities, which the Humanitarian Coordinator ultimately endorsed in consultation with the UHF Advisory Board.

7.2 Allocation Timeline

Step	Phase	Responsible body	Date
Step 1. ALLOCATION	Launch of Allocation Strategy	HC, OCHA/HFU	**23-Dec-2024**
	Allocation launch - initiation of a Call for Proposals	HC, OCHA/HFU	**17-Jan-2025**
Step 2. PROPOSAL DEVELOPMENT & SUBMISSION	Development of project proposals	Partners	**Until 07-Feb-2025**
	Deadline for submission of project Proposals.	Partners	**07-Feb-2025**
Step 3. PROPOSAL REVIEW	Preliminary screening of project proposals	OCHA/HFU	**10-11-Feb-2025**
	Review of project proposals and the HC endorsement	OCHA/HFU, Review Committees, Partners, HC	**17-Feb-2025**
	Final technical and financial review of project proposals (rolling basis)	OCHA/HFU, Clusters, Partners	**From 24-Feb-2025**
Step 4. PROPOSAL APPROVAL	Final approval by HC and Grant Agreement generation (rolling basis)	HC, OCHA/HFU	**From 03-Mar-2025**
	Grant Agreement signature by HC, Partner, and OCHA EO (rolling basis)	HC, Partners, OCHA EO	**From 04-Mar-2025**
Step 5. DISBURSEMENTS	Release within 10 working days of EO signature (rolling basis)	OCHA/CBPF Section	**From 17-Mar-2025**

Section 8: Contacts and Complaints

8.1 Key Contacts

Up-to-date **contact details of cluster and inter-sectoral working group coordinators** can be found at this link.

All **correspondence regarding the UHF**, including questions on OneGMS, should be sent to ocha-uhf@un.org.

8.2 Complaints and Feedback Mechanism:

Complaints and feedback from stakeholders regarding the UHF allocation process should be sent to feedback-uhf@un.org. The OCHA Ukraine Head of Office will receive, address, and refer any critical issues to the HC for decision-making.

Section 9: List of Annexes

Annex 1: IASC Gender with Age Marker Information Sheet (English and Russian)
Annex 2: UHF proposal guidance on disability (English and Ukrainian)
Annex 3: Guidance for developing UHF project proposals (English and Ukrainian)
Annex 4: UHF budget preparation guidelines (English and Ukrainian)
Annex 5: UHF Sub-IP budget and BoQ templates (English)